ANX-05 | Therapist guide only

Learning without requiring complete certainty

ANX-05 | A flexible therapist framework, guided by assessment and agreements.

Purpose

Compare an expectation with an outcome in a specific case, identify conditions and explanations, and consider what to take forward. Learning and action can occur with anxiety or uncertainty present. Their disappearance is not promised.

Before use

Use a record or description of an activity already planned with the therapist. Return to the original expectation without changing it to obtain a desired result. If unattempted, explore barriers; do not fill in imagined results as events.

A possible working sequence

Open with SES-S01 as needed and close with SES-S02, feedback and a clear plan.

1. Recall the expectation and plan as defined beforehand.
2. Describe actions and outcomes, separating observation, sensation and interpretation.
3. Identify similarities and differences and possible explanations, including context and support.
4. State learning limited to the data and allow for what remains unknown.
5. Choose next steps separately after reassessing wishes, conditions and suitability.

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Examples for exploration, not ready-made tasks

Examples are fictional. A real client's circumstances, needs and wishes take priority.

Group	A conversation example
Children	A child expected to understand none of a game, but understood part and asked for help elsewhere. Retain both observations without deciding help is unnecessary.
Adolescents	A teenager started a draft but still worried about continuing. Learn that starting was possible under certain conditions without concluding the rest will be easy.
Adults	An adult expected no enjoyment while worried. She describes a moment of interest alongside tension; this is specific information, not a promise for next time.
Older adults	A person attended part of a gathering with chosen help. Explore what was possible and unsuitable, without attributing everything to age or willpower.

A key distinction

One outcome is not proof for all cases. Do not make learning a quest for absolute assurance or a requirement for positive interpretation. Acknowledge real difficulties and resulting needs instead of erasing them with reassurance.

A question for shared reflection

What conclusion fits the available information, what other explanations are reasonable and what can remain open without deciding now?

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Individual adaptation and scope

An age label does not replace assessment of development, communication, context and preferences.

Group	A possible approach
Children, approximately ages 6 to 11	Use a concrete situation and one question at a time. The therapist can write spoken answers and check understanding. Hear the child's voice too. Adult roles, sharing and consent depend on the setting and need; do not promise absolute confidentiality.
Adolescents, approximately ages 12 to 17	Choose a personal focus and recording method that respects privacy. Explore setting, demands and relationships. Explain sharing and consent limits, and arrange help without making practice a test or demand for obedience.
Adults	Include roles, culture, language, circumstances and resources as described. Do not assume time, money or support is available. Shared choice does not replace clinical suitability assessment.
Older adults	Use respectful adult language, comfortable pace and spacing. Ask about preferred completion methods and help without assuming reduced ability or family dependence. Life experience and preferences inform the work.

Checking understanding and choice

Ask the client to explain their understanding and choice in their words. Shorten, leave fields blank or answer aloud. If questions are unsuitable or burdensome, adapt the approach rather than require completion.

This unit's specific boundary

Accepting uncertainty does not mean abandoning relevant information, necessary protection or care for a real problem. Do not ask clients to claim they can tolerate any situation. Learn and choose according to the case and treatment framework.

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Practice, follow-up and professional review

Practice is chosen jointly; the sheet does not automatically authorize action.

Before practice

ANX-05-P records use of learning in another situation chosen with the therapist, only if suitable. Similarity does not guarantee suitability. Assess every new activity and its conditions beforehand; difficulty does not automatically increase or support decrease.

Back in the session

Review what was possible, learning and remaining questions. Retain the direction, return to mapping or change approach. ANX-06 maintenance considers goals and functioning over time and does not require absence of anxiety.

Production and review status

Original draft guide and sheets. Professional review for each age and a rights review are needed before distribution. Completed writing is not clinical approval; the kit has not been validated or tested for effectiveness.

General sources for further reading

NICE CG113 — adults
Professional framework for adult generalized anxiety: assessment, functioning and work by trained therapists. It does not validate these six units or their use with children.
https://www.nice.org.uk/guidance/cg113/chapter/Recommendations

NIMH — child anxiety, 2024
Research background on CBT for children; the study used a different protocol. It does not approve this wording or these age adaptations.
https://www.nimh.nih.gov/news/science-updates/2024/cognitive-behavioral-therapy-alters-brain-activity-in-children-with-anxiety

Consulted: 03 Nov 2026. Background sources only, without permission to copy forms or validation of the kit.

Reviewer's name, role, date and versions reviewed:

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